



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض ربيعاً : مديحة سالم عبدالجواد ابوزيد

العنوان : ---

المحافظة : الجيزة

رقم البطاقة : 25911242101585 رقم التليفون : 01128204386 تاريخ تسجيل الإستمارة : 04-10-2026

العمر (تاريخ الميلاد) : 24-11-1959 المهنة : بدون عمل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... /د. /..... /د. /..... /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: مديحة سالم عبدالجواد ابو زيد National ID: 25911242101585	
---	---

تقرير طبي

السيد
تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

مديحة
سالم عبدالجواد ابوزيد
قرية الكدايه
مركز اطفح - الجيزه

٢٠٠٩/١١/١١

٢٠٩ ١١ ٢٤ ٢١ ١٥ ٨٥

LD4186939

DMS

محمد حنظل

٢٠٢٥/٠٥ ٢٠٩ ١١ ٢٤ ٢١ ١٥ ٨٥
ريه منزل
انثى مسلمة
البطاقة سارية حتى ٢٠٢٢/٠٥/١٦

١



رسوم الاستعلام 20 جنية

جهة الاصدار : فرع الجيزة

تاريخ الاصدار : 2026-4-8

الى من يهمه الامر

الهيئة العامة للتأمين الصحي تعمل من خلال قواتين منظمة للانتفاع الخاضعين لقانون التأمين الاجتماعي رقم 79 لسنة 1975 المعدل بالقانون 25 لسنة 1977 "القوى العاملة والمعاشات والارامل " و كذلك قانون 99 لسنة 1992 الخاص بطلاب المدارس و قانون رقم 86 لسنة 2012 " الاطفال دون السن المدرسي " وقانون رقم 23 لسنة 2012 الخاص بالمرأة المعيلة .
وطبقا لقاعدة البيانات المرسله من التأمينات الاجتماعيه
وطبقا لقاعدة البيانات المرسله من الضمان الاجتماعي
والبحث تبين أن

السيد (ة) مديحه سالم عبد الجواد بطاقة رقم 25911242101585

تليفون رقم: لا يوجد

وطبقا الى خطاب من التأمينات الاجتماعية يفيد عدم الانتفاع
يعتبر من الفئات غير المنتفعه بالتأمين الصحي صدر هذا الخطاب دون اذن مسئولية عن الهيئة العامة للتأمين الصحي حال خضوع المواطن لقانون التأمين الصحي الشامل
وشكرا

تاريخ الطباعة : 2026-4-8



مسلسل 1944003



الخطاب ساري لمدة ثلاثة أشهر من تاريخ الإصدار إلا في حالة تغيير حاله المواطن من قبل الهيئة القومية للتأمينات الاجتماعية أو
وزارة التضامن الاجتماعي أو وزارة التربية و التعليم

الإدارة المركزية لتنظيم المعلومات و التحول الرقمي

Name: مديحه سالم عبدالجواد ابوزيد			
MR. : 10505			
Detailed Medical Report			
			السيد
تحتية طبية وبعد,,			
		إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
		رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient presents with a history of hypertension and post-menopausal right breast cancer, classified as CT4b N1 Mx with a retrocaval lymph node measuring 13 × 13 mm. Imaging and pathology indicate multifocal malignant masses with axillary lymph node involvement, estrogen receptor positivity (ER 8/8), progesterone receptor positivity (PR 4/8), HER2 amplification, and a high Ki-67 index (22 %). The patient has right breast edema and peau d'orange with retro-aleolar masses measuring 3 × 3 cm and 2 × 2 cm, and axillary lymph nodes measuring 2 × 2 cm. The current systemic therapy plan includes neoadjuvant chemotherapy with taxol 130 mg/m² (10 % dose reduction) and carboplatin AUC 2, with a body-surface-area-adjusted weight of 91 kg, height 155 cm, and a serum albumin of 1.9 g/dL. Approval for phesgo is pending. The patient was evaluated in the internal medicine clinic for cycle 1 week 2 of taxol and carboplatin, with pharmacy and clinic visits scheduled for week 3 and week 2 of cycle 2, respectively, and laboratory work performed. On examination during week 4 of taxol 130 mg/m² and carboplatin AUC 2, the patient continues to have right breast edema. The neoadjuvant chemotherapy regimen remains unchanged, and the breast-specific staging is still pending. A PET-CT performed on 20 August 2026 shows no mediastinal lymphadenopathy or osseous lesions; the scan is free except for two retro-aleolar and right-lateral breast masses with an SUV of 2. The patient is on week 5 of taxol 130 mg/m² and carboplatin AUC 2, with week 6 approval of phesgo pending. The patient has completed 5 weeks of taxol 130 and carboplatin AUC 2.			
تحريرا في ,			
		التاريخ : 19-09-2026	الوقت :
		dina makram	الطبيب المعالج :
يعتمد ,			

Patient's Name: Madiha Salem Abdeljawad
Date: 2026-06-10
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

Known case of breast CA.

I- CHEST:

- o Two right breast lesions measuring 34 x 19 mm and 22 x 16 mm with few right axillary lymph nodes largest measuring 17 x 14 mm for sono-mammography.
- o No pulmonary nodules, masses or cavitary lesions.
- o No consolidation or collapse.
- o Retrocaval lymph node is seen measuring 13 x 13 mm.
- o No pleural or pericardial collection.
- o Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- o Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- o Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- o Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- o The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- o Normal appearance of the pelvic organs with no pelvic masses or collections.
- o No abdominal or pelvic lymphadenopathy.
- o No ascites.
- o Mild anterior wedging of D12 vertebral body.

Opinion:

- *Two right breast lesions with right axillary lymphadenopathy for sono-mammography.*
- *Retrocaval lymph node for PET-CT assessment.*

Dr.sherif Wagdy

Dr.sherif Wagdy

mm/dd/yyyy

MDT FORM

Name: <u>عليه سالم عبد الجواد الوكيل</u>	MRN: <u>10505</u>
Age: <u>67 Y</u>	ID: <u></u>
Menopausal Status: <u>POST</u>	Phone: <u>0112620395</u>
NRS SCORE: <u></u>	Marital Status: <u></u>
BMI: <u></u>	Family History: <u></u>
CO Morbidity: <u>DM HTN</u>	Reason Of MDT: <u>NEW CASE</u>

Clinical Data:

BX: IDC G2
ER 8/8 PR 4/8 HER -2 SI KI67 35%

Sonomammography:

1) SM
THICKENED RT BREAST SKIN WITH TWO IRREGULAR SHAPED LESIONS IN LIQ WITH SPECULATED MARGIN WITH DISTORTION . ANOTHER IRREGULAR MASS IN LIQ IN POST THIRD WITH DISTANCE IN BETWEEN 5.2 CM
TWO MASSES MEASURE 13*17 MM , 12*11 MM
3 SUSCIPIOUS INTRAMAMMARY LN 15*12 MM
2) CSM
LOWER CENTRAL & LOQ 3 SPICULATED DENSE MASS LESIONS , HETEROGENOUS ENHANCING MASSES EXTEND FROM ANTERIOR TO POSTERIOR THIRD OF BREAST 2.3*2.2*1.7 CM , 3.2*1.7*2.5 CM & 1.5*2.6*1.5 CM RESPECTIVELY
THE TWO ANTERIOR LESIONS SURROUNDED BY 2

Mets Workup:

CT CAP
FREE EXCEPT RETROCAVAL LN 13*13 MM

History:

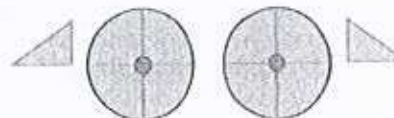
Recommendations:

MDT Outcome & Plan

AS MULTI CENTERIC AND SKIN THIKING ,
MICROCALCIFICATION IN SM FOR NACTH THCP

R

L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
<u>د. محمد عبد الوكيل</u>	<u>د. محمد عبد الوكيل</u>				

دكتور
عباس عبدالمنعم جبر
دكتوراه الاشعة التشخيصية
والموجات فوق الصوتية

PATIENT NAME السيدة / مديحة سالم عبدالجواد
DEAR PROF.DR ا.د. / هشام فرجاني
DATE: 1 / 4 / 2026

BOTH BREAST ULTRASONOGRAPHY REVELD :

- Right lower quarter , mild lobulated outline hypoechoic solid lesion 15x12mm. no calcification , mild nipple retraction
- Both breast showing normal echogenic fatty pattern Normal shape areola nipple and subcutaneous tissue thickening , no micro or macro calcification .
- No suspicious smooth solid lesion.
- No cystic changes .
- Mild left dilation of the intra mammary ducts.
- No pathological enlarged axillary lymph nodes .

IMPRESION:

- Right lower quarter breast suspicious mass 15x12mm , BIRADS 4 .

BIRADS 1	NAD
BIRADS 2	Benign
BIRADS 3	Probably Benign
BIRADS 4	Suspicious Malignancy
BIRADS 5	Highly Suggestive Malignancy

Much Obligated

العنوان: عمارات اطفح الجديدة بجوار الادارة التعليمية ومجنس مدينة اطفح
المواعيد من ٩ صباحا الي ٦ مساءا يوميا عدا يوم الجمعة 38411931-38416250

Patient ID: 94529	Patient Name English: Madinah Salem Abdgawad Abozaid
Age: ٤٦ سنة, ٣ شهر	2026/04/09 11:27:33 AM :Report Date

Women's Health Outreach Program
Bilateral Full field Digital Mammography

Clinical summary: Right breast lump.

Technique

Medio-lateral oblique and CC views. High resolution ultrasound.

Right breast

- Scattered fibro glandular densities (ACR B).
- Lower central middle and posterior 1/3s three irregular shaped spiculated lesions with intralesional pleomorphic grouped microcalcifications, overall measurement about 10.7X4.6 cm.
- **Ultrasound** demonstrated lower 6-7 o'clock three irregular shaped non-parallel hypoechoic vascular lesions as follows:
 - **Index** lesion seen at 6 o'clock zone b measuring about 3.6X1.6 cm (7 cm from the nipple and 0.2 cm from the skin).
 - Adjacent medial one (distance to **index** 1.4 cm) measuring about 1.4X1 cm (4 cm from the nipple and 0.5 cm from the skin).
 - Another zone c lesion (distance to **index** 8 cm) measuring about 1.7X1.2 cm (13 cm from the nipple and 0.5 cm from the skin).
- Mild skin thickening. No dilated retro-areolar ducts.
- Multiple more than three enlarged axillary level I and II lymph nodes, the largest one seen at the axillary tail measuring about 1.7X1.2 cm, with muffled hilum likely infiltrated.

Left breast

- Scattered fibro glandular densities (ACR B).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin and nipple/areolar complex.
- Mild dilated anechoic retro-areolar ducts reaching 0.46 cm.
- Preserved axillary LNs.

Impression:

- Right breast lower central multiple malignant looking lesions with ipsilateral axillary infiltrated LNs (BIRADS 5), US guided biopsy taken at the same setting.
- Left breast (BIRADS 2): mild ductectasia.

Dr. Amira Shaheen, MD

Patient's Name: Madiha Salem Abdeljawad
Date: 2026-06-10
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

BIATERAL MAMMOGRAM AND BREAST ULTRASONOGRAPHY REVEALED:

Indication: Palpable right breast mass with no available pathological data.

- **Breast density:** Predominantly fatty breast parenchyma... Matching ACR a.
- Thickened right breast skin with two irregular shaped lesions are seen in the LIQ with speculated margin are seen in the lower inner quadrant of the right breast, associated with inbetween area architectural distortion .Another irregular shaped mass is seen in the LIQ in the posterior third with distance inbetween 5.2 cm .
- No detected suspicious mass density, architectural distortion or pathological micro-calcification in the **left breast**.
- Normal skin and nipple on the left side ..

By complementary breast ultrasound:

- Homogeneous (predominantly fatty) echo-texture of both breasts.
- Two irregular shaped non circumscribed margin hypoechoic masses are seen at 5 oclock postion at the areolar margin measuring (13 x 17 mm) with nearby mass (15 mm distance) that measures (28 x 18 mm) .
- Another irregular shaped mass is seen at 5 oclock ,zone 3 ,measuring (12 x 11 mm) .
- Normal left skin and nipple.
- No detected suspicious solid or cystic masses in the left breast.
- Three suspicious intramammary right lymph nodes, largest measures (15 x 12 mm) .

CONCLUSION:

- Right breast : Right LIQ multifocal highly suspicious masses BI-RADS 5 category.
- Left breast :Normal

Prof.MONA ZAKY

Patient ID: 95333	Patient Name English: Madiah Salem Abd Algawad Abo Zaid
Age: 67 years	2026/05/12 01:33:41 PM :Report Date

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: A case of right IDCII

CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: mild nodular background enhancement
ACR: b (scattered fibro-glandular densities)

LEFT BREAST:

- **Low energy (Non contrast) images:** show no masses, asymmetry or distortion or suspicious microcalcifications.
- Scattered benign calcifications
- **Recombined (Post contrast) images:** show, no pathological uptake.
- Normal skin / nipple / areolar complex.

RIGHT BREAST:

- **Low energy (Non contrast) images :**
 1. Lower central & LOQ 3spiculated irregular dense mass lesions (with intralesional microcalcifications) extend from anterior to posterior third of the breast with skin thickness and nipple retraction .
 2. Enlarged axillary tail & UOQ intramammary nodes.
- **Recombined (Post contrast) images:**
 1. Lower central & LOQ 3 spiculated moderate heterogeneously enhancing masses extend from anterior to posterior third of the breast, they measures 2.3x2.2x1.7cm 3.2x1.7x2.6cm & 1.5x2.6x1.5cm respectively from anterior to posterior.
 2. The two anterior lesions seen surrounded by 2 small enhancing masses (0.5cm) all forementioned lesions shows segmental distribution reaching the nipple, the **overall coverage area of the process** is 13x2.3x2.5cm .
 3. UOQ intramammary enhancing lymph node.
- **Complementary ultrasound:**
 1. 6:00 2 zone A&B Irregular masses with intraductal extension reaching the nipple
 2. 7:00 zone C irregular hypoechoic mass.
 3. 9:00 about 2 pathological intramammary lymph nodes.
 4. Level I more than 3 pathological discrete lymphnodes, largest shows indenting hilum and thickened cortex with overall measurement 1.5x1.4 with 1.1cm cortical thickness

IMPRESSION:

LEFT BREAST: (BIRADS2 =benign)

RIGHT BREAST: (BIRADS 6=pathologically proved malignancy):lower central &LOQ pathological enhancing masses as described

Dr. Marwa Ramzy, MD. FRCR

Patient Name	الاستاذة / منيرة سالم عبدالعزيم ابو زيد	Request No	13005800
Patient No.	25911242101585	Request Date	09/04/2026
Path No.	958/26	Verification Date	21/04/2026
Sex	Female	Financial Category	جوازات - مصري - حائز
Age	66 Y	Department	Out Patient
Referring Doc		Registered by	Ali mostafa

IMMUNOHISTOCHEMISTRY REPORT

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within > 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Wafaa Ibrahim Ghoneim

Dr. Heba El Shafey

Verified by

Dr. Duaa Mohamed

Printing Date : 26/04/2026 09:36:06

Page 2 of 2

Printed By : marwa mohamed youss



عنوان (4) - الدور الأول - مصر رقم (987) - طريق النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor Building No. 987, Nile Citiche - Maat El Qadima - Cairo, Egypt

Patient Name : (الاستاذ / مديحة سالم عبدالجواد البوزيد)
Patient No. : 25911242101585
Path No. : 958/26
Sex : Female
Age : 66 Y
Referring Doc :

Request No. : 13005800
Request Date : 09/04/2026
Verification Date : 21/04/2026
Financial Category : جوماتك روسي - مائز -
Department : Out Patient
Registered by : Ali mostafa

Pathology Report

Clinical Data: Right breast lower central 6 O'clock ill defined mass (BIRADS 5).
Nature of Specimen: Tru-cut biopsy, collected on 9/4/2026, at 12:00 pm. Cold ischemia time is 20 seconds.

Gross Examination:

- Six grey white tissue cores, the longest is 1.0 cm, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
- * Ductal carcinoma in situ: Focal intermediate & highgrade DCIS (Solid & cribriform patterns).
- * Perineural invasion: Absent.
- * Angioinvasion: Absent.

Diagnosis:

Right breast mass. Tru-cut biopsy.

- Invasive duct carcinoma, Grade 2.
- Focal intermediate & high-grade ductal carcinoma in situ (Solid & cribriform patterns).
- For further IHC (ER, PR, Her2, Ki67)

Signature

Dr. Walaa Ibrahim Ghoneim

Dr. Heba El Shafeey

Verified by :

Dr. Duaa Mohamed

Printing By : marwa mohamed yous

Printing Date : 26/04/2026 09:36:01

Page 1 of 1



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - مستشفى قصر العبدية - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Ndx Corniche - Min El Qadima - Cairo, Egypt

Patient Name	: الأسماء / منيرة محمد عبد الجواد أبو زيد	Request No.	: 13005800
Patient No.	: 25911242101585	Request Date	: 09/04/2026
Path No.	: 958/26	Verification Date	: 21/04/2026
Sex	: Female	Financial Category	: جواز سفر - مصري
Age	: 66 Y	Department	: Out Patient
Referring Doc.		IHC No.	:
		Registered by	: Ali mostafa

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast lower central 6 O'clock ill defined mass (BIRADS 5) diagnosed "Invasive duct carcinoma grade 2 with focal intermediate & high grade DCIS (Solid & cribriform patterns)"
 Nature of Specimen: Tru-cut biopsy, collected on 9/4/2026, at 12:00 pm. Cold ischemia time is 20 seconds.

Technique

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9) & SISH.

Results:

- * IPT (All controls showed appropriate reactivity):
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Moderate nuclear reaction in 5% of tumor cells (score 4/8).
- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Amplified with clusters.
- Ki67%: 35%.

Conclusion:

- Right breast mass, Tru-cut biopsy:
- Invasive duct carcinoma, Grade 2.
 - Focal intermediate & high grade ductal carcinoma in situ (Solid & cribriform patterns)

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 4/8).
- Her2: Equivocal (score 2+).
- SISH: Amplified with clusters.
- Ki67%: 35%.

Verified by
 Dr. Duaa Mohamed

Printing Date : 26/04/2026 09:36:06

Page 1 of 2

Printed By : marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - مستشفى النيل - مصر الجديدة - القاهرة - مصر
 Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Msc El Qadima - Cairo, Egypt

Report No: 04539	Patient Name English: Madhavi Suresh Abdegawad
Ref: P. Sha FFA01	Version:
	1/26/2019 11:27:21 AM Report Date

Women's Health Outreach Program
Bilateral Full field Digital Mammography

Clinical summary: Right breast lump.

Technique

Medio-lateral oblique and CC views. High resolution ultrasound.

Right breast

- Scattered fibro glandular densities (ACR B).
- Lower central middle and posterior 1/3s three irregular shaped spiculated lesions with intralesional pleomorphic grouped microcalcifications, overall measurement about 10.7X4.6 cm.
- **Ultrasound** demonstrated lower 6-7 o'clock three irregular shaped non-parallel hypoechoic vascular lesions as follows:
 - **Index** lesion seen at 6 o'clock zone b measuring about 3.6X1.6 cm (7 cm from the nipple and 0.2 cm from the skin).
 - Adjacent medial one (distance to **index** 1.4 cm) measuring about 1.4X1 cm (4 cm from the nipple and 0.5 cm from the skin).
 - Another zone c lesion (distance to **index** 8 cm) measuring about 1.7X1.2 cm (13 cm from the nipple and 0.5 cm from the skin).
- Mild skin thickening. No dilated retro-areolar ducts.
- Multiple more than three enlarged axillary level i and ii lymph nodes, the largest one seen at the axillary tail measuring about 1.7X1.2 cm, with muffled hilum likely infiltrated.

Left breast

- Scattered fibro glandular densities (ACR B).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin and nipple/areolar complex.
- Mild dilated anechoic retro-areolar ducts reaching 0.46 cm.
- Preserved axillary LNs.

Impression:

- Right breast lower central multiple malignant looking lesions with ipsilateral axillary infiltrated LNs (BIRADS 5), US guided biopsy taken at the same setting.
- Left breast (BIRADS 2): mild ductectasia.

Dr. Amira Shaheen, MD

Name : Madiha Salem Abdeljawad Abu Zeid
Branch : Dokki
Medical No. : 443624
Date : 20-08-2026 07:42
Ref.Physician Prof : Goustav Roussey Egypt

Whole Body (FDG) PET/CT Scan

Clinical question:

- 66-year-old lady with history of Rt. breast mass pathologically proven as invasive duct carcinoma grade 2.
- PET/CT is requested for status evaluation.

Procedure:

- Dose/Radiopharmaceutical: 4 mCi ^{18}F -FDG administered via a peripheral upper limb vein.
- Blood glucose level: 116 mg/dL.
- Imaging was performed on an integrated PET/CT scanner approximately 60 minutes after injection of the radiopharmaceutical.
- CT without contrast images done for attenuation correction and an anatomic localisation were obtained followed by PET images from the vertex of the skull to the calf region.
- PET/CT images were reviewed in the transaxial, coronal and sagittal planes.
- The Standardised Uptake Value, maximum variant (SUV_{max}) was calculated within regions of interest whenever needed.

Findings:

- FDG avid right breast two retroareolar and LIQ soft tissue lesions, the largest measuring 2.8x1.6 cm and eliciting SUV max up to 2. Associated overlying skin thickening also noted.
- Unremarkable appearance of the left breast.
- Few small Rt. axillary LN.s with no significant FDG avidity.
- No other enlarged or dense tracer fixing mammary related LN.s.
- No lung nodules, masses or parenchymal infiltrates.
- No pleural or pericardial effusion.
- Normal tracer distribution over the myocardium.

Head and neck:

- The brain looks normal with no abnormal focal metabolic activity.
- (N.B: it may kindly be noted that some brain lesions not to be apparent on PET/CT and MRI is better for local assessment, if clinically warranted).
- Normal FDG uptake pattern is seen at other soft tissues, glandular structures, oropharynx, larynx and salivary glands.
- No adenopathy or nodal hypermetabolism in the neck.

Name : Madiha Salem Abdeljawad Abu Zeid
Branch : Dokki
Ref. Physician Prof : Goustav Roussey Egypt

Date : 20-08-2026 07:42
Medical No. : 443624

Abdomen and pelvis:

- Average-sized liver with no focal lesions or FDG-avid foci. No IHBR dilatation could be detected.
- No significant pathologically enlarged or metabolically active pelvi-abdominal lymph nodes.
- Normal spleen, pancreas, kidneys, suprarenals and pelvic structures.
- No ascites.

Musculoskeletal:

- No blastic or lytic bony lesions could be detected all over the surveyed skeleton.

Impression:

Whole body (FDG) PET/CT study is showing:

- FDG avid right breast two retroareolar and LIQ soft tissue lesions, matching with the known active tumour biology.
- Free rest surveyed body from FDG avid focal lesions.
- Follow up FDG PET/CT would be advisable if clinically justified.

Best regards,

Dr. Mohammed Akl, M.Sc

Dr. Alaa Nagi, MD
Nuclear Medicine